

إسم المريض رابعياً : مديحه عياد عبده جرجس

المحافظة : القاهرة الحي / المدينة : دار السلام

تاريخ تسجيل الإستمارة : 05-10-2026

رقم البطاقة : 26902240100969 رقم التلفون : 01125521455

العمر(تاريخ الميلاد) : 24-02-1969 المهنة : ربه منزل

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-
الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة
نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (-----) المقر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي علاج هرموني خط ثاني مثبتات الاروماتيز [14161]

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد کیلانی

الاسم: د/ندی حسن

الاسم: د/نرمين رمضان

الوظيفة: ----- التوقيع:

الوظيفة: ----- التوقيع:

الوظيفة: ----- التوقيع:

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.

كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

----- / 2 ----- / 2 ----- / 2.1 ----- / 2.1

قرار اللجنة

التشخيص النهائي :

خطـة العـلاج :

المستشفى :

جهة العلاج :-

ملاحظات : ---

ختم

شعار الجمهورية

مدير عام

مديرية الشئون الصحية

رئیس

المجلس الطبي المتخصص

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Name: مديحه عياد عبده جرجس National ID: 26902240100969	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حمید بالثدي و تحتاج الي Second Line Hormonal Treatment Aromatase Inhibitor
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التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



٢٠٢١/٠٦ ٢٦٩٠٢٢٤٠١٠٠٩٦٩
رؤية منزل
أنشئ مسيحية منزلة
مسعد يعقوب مسعد معوض
الليطافة مسارية حتى ٢٠٢٩/٠٦/٠٦



Name: <u>مديحه عياد عبده جرحس</u>		
MR. : <u>6892</u>		
Detailed Medical Report		
		السيد
تحتية طبية وبعد,,		
	بشأن تقرير طبي للمريض	إيماء للطلب المقدم من
	رقم طبي:	بقسم:
ننشرف بأن نوضح فيما يلي تقريراً عن حالته		
	تاريخ وساعة الخروج :	تاريخ وساعة الدخول :
وبعد الكشف والفحوصات والمتابعة وجد انه		
MDT 9/2023: The patient was diagnosed with T2 N0 N0 disease and is scheduled for adjuvant therapy with aromatase inhibitor (AI) plus zoledronic acid (ZOMETA). The plan includes a 12-month course of FEMARA and additional supportive measures as requested. T2N0M0: In January 2026, the patient underwent a BIRADS 1 assessment of the surgical margin (SM). The patient is to receive adjuvant aromatase inhibitor therapy (ARAMIDEX) for a duration of three months. For adjuvant ARAMIDEX: Supportive care has been ordered along with laboratory monitoring for zoledronic acid therapy. Adjuvant: The patient will continue ARAMIDEX for a total of three months with supportive care. Neck ultrasound is scheduled, and the next visit is planned for 1/11/2026.		
تحريراً في ,		
	الوقت :	التاريخ : 20-09-2026
	نهي سعيد علي محمد	
الطبيب المعالج :		
يعتمد ,		



Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان



MDT form

A- Patient ID:

1- P/BME : ملاحظة طبية كبدية جرحية

Sex:

3- Medical number:

Male

83124013

Old Med :

398/2023

2- ID number: 26902240100969

Patient Age : 54 Y



Family history

Phone

01021051072

BMI

REASON OF MDT REMDT

marital status

MARRIED

NRS Score

GROUP

A

Date of MDT 2023-07-29

Co-morbidity

HTN
HYSTRECTOMY
(FIBROID)

Menopausal
status

Clinical data

Breast Imaging :

Mets Workup :

CT CAP FREE

BX: IDC GII	ER: 8/8	PR: 6/8	Her 2 Neu : 0	SISH : 25	KI67 : 25
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T 3	N 0	M 0
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Sonmammography

PATIENT HISTORY

POST MENOPUSAL RT BC CT2 N0M0

PT RECEIVED 3 AC+3 TAXOL LAST ONE ON 26/6/2023

LAST MDT 12/7/2023-->FOR ASSESSMENT MAMOGRAPHY TO BE REVISED AT SURGURY CLINIC
THEN CONSIDER WIRE LOCALIZATN THEN WLE+SLNB

BSM ON NCI SHOWS PATHOLOGICAL DIFFUSE MICROCALCIFICATION.

MDT OUTCOME & PLAN

FOR RIGHT MASTECTOMY & SLNB (IF AVAILABLE)

SIGNATURE

MEDICAL ONCOLOGY	د. منير الشوكري	RADIOLOGY	د. شوقي ممدوح
SURGERY	د. منة إبراهيم الانلي	PATHOLOGY	
Psychiatric		Nutrition	

Patient Name: Madiha Ayad Abdou Gerguis	Date : 22/02/2023
Exam : C.T. ABDOMEN & PELVIS WITH CONTRAST	Age : 54 Year(s)
Medical ID:	

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS: -

Findings:

I Chest:-

- Right breast retroareolar irregular lesion measures 1.7 x 2.4 cm.
- Clear both lung fields. No sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.
- Heterogenous enlarged thyroid gland for neck US assessment.

II Abdomen & pelvis:

- The liver is of average size with homogenous CT texture. No focal lesions or contour irregularities.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- No suspicious lytic or sclerotic lesions

Opinion:



Patient Name	: الأستاذة / مديحة عياد عبده جرجس	Request No.	: 13009856
Patient No.	: 26902240100969	Request Date	: 05/02/2023
Path No.	: 347/2023	Verification Date	: 16/02/2023
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 53 Y	Department	: Out Patient
Referring Doc.	:	IHC No.	:
		Registered by	: marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Right breast retro-areolar malignant looking solid mass lesion.
Nature of Specimen: Tru cut biopsy, collected at 05/02/2023, 11:30 am.

Gross Examination:

Four grayish white and fatty tissue cores up to 1 cm, totally submitted.

IHC technique: Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for ER, PR, HER2, ER clone (1E2), PR clone (SP1), HER-2 clone (4B5), & Ki67 clone (30-9).

Microscopic Examination:

- * Histological type: Invasive duct carcinoma with microcalcification.
- * Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+2) Grade II.
- * DCIS: No.
- * Angioinvasion: Couldn't be detected in the received biopsy
- * IPT (Positive control showed appropriate reactivity) :
- * ER: Strong nuclear reaction in 95% of tumor cells (Allred score 8/8)
- * PR: Strong nuclear reaction in 15% of tumor cells (Allred score 6/8)
- * HER2: No membrane reaction in tumor cells (score 0)
- * Ki 67: 25% nuclear reaction of tumor cells

Diagnosis

Right breast retro- areolar mass, Tru cut biopsy:
Invasive duct carcinoma, Grade II.

IPT:

- * ER: Positive (score 8/8).
- * PR: Positive (score 6/8).

Verified by :

Dr.Heba Hasan

Printing Date : 18/02/2023 08:58:27

Page 1 of 2

Printed By : marwa mohamed yousse



Patient Name : الأستاذة / مديحة عباد عبده جرجيس
Patient No. 26902240100969
Path No. 347/2023
Sex Female
Age 53 Y
Referring Doc.

Request No. 13009856
Request Date 05/02/2023
Verification Date 16/02/2023
Financial Category اورام دار السلام - نفقة الدولة
Department Out Patient
Registered by .marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

- * HER2: Negative (score 0).
- * Ki67: Positive in 25% of tumor cells

Signature
Prof. Dr. Amal Hareedy
Dr. Heba Hassan

Verified by :
Dr.Heba Hasan

Printing Date : 18/02/2023 08:58:27

Page 2 of 2

Printed By : .marwa mohamed yousse

Name : Mrs. Madeha Ayad Abdou Guirgia
Date : 17-07-2023

Code : R2922104

MR MAMMOGRAPHY OF BOTH BREASTS:

CLINICAL SUMMARY:

A known case of right breast carcinoma, treated by neo-adjuvant chemotherapy and radiotherapy, for pre-operative assessment

IMPRESSION:

LEFT BREAST: (BIRADS 1)

Normal study

RIGHT BREAST: (BIRADS 6)

Residual paraareolar neoplastic mass as described, for further clinical assessment

TECHNIQUE:

Multiplanar imaging was performed on a 1.5T magnet with a dedicated breast coil using the following sequences:

- Axial T1, T2 IR
- Sagittal T2
- Subtraction of pre & post contrast 3D dynamic axial Fat suppressed FLASH in 1.5mm slice thickness after 0.1mmol/kg Gd-DTPA with MIP image reformatting

FINDINGS:

LEFT BREAST:

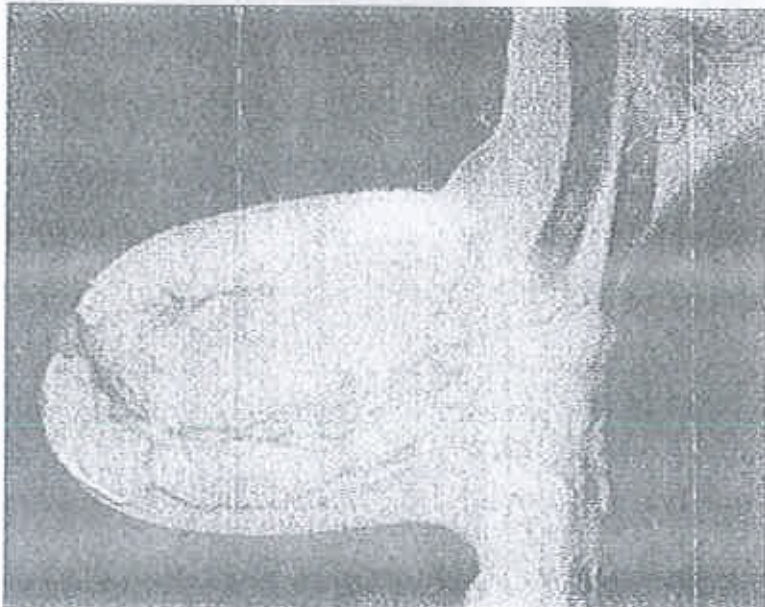
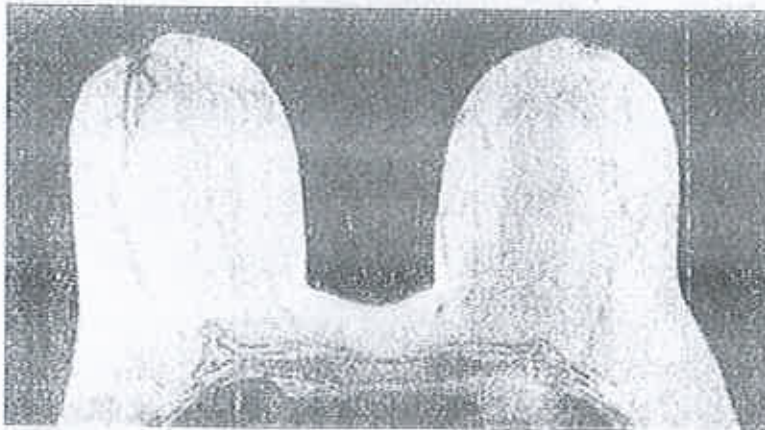
- On the pre-contrast series, no abnormal signal is seen.
- After injection of contrast, no rapidly enhancing masses or other suspicious enhancement patterns were observed to suggest invasive malignancy.
- Normal skin/nipple/areolar complex.
- No pathological axillary lymph nodes.

Name : Mrs. Madaha Ayad Abdou Guirgis
Date : 17-07-2023

Code : R2922104

RIGHT BREAST:

- An ill defined spiculated mass is seen in the paraareolar region, reaching the nipple and extending backwards measuring collectively about 5.5x3cm, exhibiting low signal in the T1 and T2WIs and high signal in the IRWIs.
- After injection of contrast it enhanced heterogeneously.
- No definite pathological axillary lymph nodes.



Best regards,
Dr. Mahi Mokhtar

تاريخ: 23/1/2024

Patient: Madeeha Ayad Abdó

Date: 23/1/2024

^{99m}Tc -MDP BONE SCAN

Desired information: *skeletal surveying to detect multiple lesions.*

Technique: Anterior and posterior whole-body scan performed 2 hours after I.V administration of 20 mCi ^{99m}Tc -MDP;

Report:

- Warm tracer uptake is seen at the left femoral head likely of worrying nature for MRI correlation to exclude metastatic nature .
- LV5 Rt sided facet joint arthropathy .
- Degenerative changes are noticed over Dorso-lumbar vertebrae.
- Other wise , no definite hot spot that accounts for osseous deposits could be detected all over the surveyed skeleton.
- Both kidneys are normally seen.

Conclusion:

Warm tracer uptake is seen at the left femoral head of worrying nature for further MRI correlation , and close follow up .

Thanks for referral

Nuclear Medicine Unit

DR . Sherien Mahdy, MD

RD/ Mohamed Emara



Patient Name: Madiha Ayad Abdou Gerguis	Date : 11/12/2023
Exam : ultrasound ABDOMEN& PELVIS	Age : 54 Year(s)
Medical ID: 398/2023	Pat. ID: 29040

ABDOMINO-PELVIC ULTRASOUND EXAMINATION REVEALED:

Liver: -

- Average in size displaying homogenous parenchymal echopattern with smooth surface.
- No definite focal lesions or intra hepatic billiary radicle dilatation.
- Portal vein is patent and not dilated.

Gall Bladder: -

- Surgically removed
- CBD is not dilated.

Spleen: -

- average size, homogenous.

Kidneys: -

- Both are of normal size, shape, echogenicity and well differentiated with good corticomedullary differentiation.
- No stones or dilated pelvi-calyceal system
- No gross masses or space occupying lesions could be seen.

Para-aortic and pancreatic area: show no gross abnormalities.

No ascites.

Urinary Bladder: -

- Average capacity with no stones or diverticular out pouching

Uterus and adenexa; uterus is surgically removed

Opinion:

Cholecystectomy
Hysterectomy

Signature Dr . Nermien Ramadan

11/12/2023 12:56 PM



Patient Name: Madiha Ayad Abdou Gerguis	Date : 26/04/2023
Exam : C.T CHEST	Age : 54 Year(s)
Medical ID:	Pat. ID: 29040

NON-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST: -

Findings:

- Right breast retroareolar irregular lesion measures 1.4 x 2.4 cm with calcification.
- The anterior segment of the right upper lung lobe shows few areas of ground glassing.
- No sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- No CT pathologically enlarged hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- No gross cardiac abnormality. No pericardial effusion detected.
- Normal non-enhanced appearance of the thoracic aorta and the great vessels.
- The scanned skeleton shows no definite worrisome lytic or sclerotic lesions, yet DISH (Diffuse Idiopathic Skeletal Hyperostosis) is noted in the scanned spine.
- Upper abdominal cuts are unremarkable.
- Heterogenous enlarged thyroid gland with right sided calcific foci.

OPINION:

- *Mild inflammatory changes affecting the right upper lung lobe as described for clinical correlation.*
- *Heterogenous enlarged thyroid gland for U/S assessment.*

Dr. Sherif Sameh, M.D.



Patient Name: Madiha Ayad Abdou Gerguis	Date : 11/12/2023
Exam : ultrasound ABDOMEN& PELVIS	Age : 54 Year(s)
Medical ID: 398/2023	Pat. ID: 29040

ABDOMINO-PELVIC ULTRASOUND EXAMINATION REVEALED:

Liver: -

- Average in size displaying homogenous parenchymal echopattern with smooth surface.
- No definite focal lesions or intra hepatic biliary radicle dilatation.
- Portal vein is patent and not dilated.

Gall Bladder: -

- Surgically removed
- CBD is not dilated.

Spleen: -

- average size, homogenous.

Kidneys: -

- Both are of normal size, shape, echogenicity and well differentiated with good corticomedullary differentiation.
- No stones or dilated pelvi-calyceal system
- No gross masses or space occupying lesions could be seen.

Para-aortic and pancreatic area: show no gross abnormalities.

No ascites.

Urinary Bladder: -

- Average capacity with no stones or diverticular out pouching

Uterus and adenexa; uterus is surgically removed

Opinion:

Cholecystectomy
Hysterectomy

Signature Dr . Nermien Ramadan

11/12/2023 12:56 PM



Patient Name: Madiha Ayad Abdou Gerguis	Date : 11/12/2023
Exam : X-RAY CHEST	Age : 54 Year(s)
Medical ID: 398/2023	Pat ID: 29040

Plain X-ray **Chest** (PA view) revealed:

- Clear both lung fields.
- Clear both costo-phrenic angles.
- Normal cardiac size & configuration.
- Normal thoracic cage

Signature Dr . Nermien Ramadan

11/12/2023 12:57 PM